# Harrington Physical Therapy

615 Finley Road Suite 809
Pasadena, TX
Phone 281-555-4316

## Itemized Medical Bill

| | |
|---|---|
| **Patient:** | Tiffany Graham |
| **Date of birth:** | September 3, 1996 |
| **Insurance claim:** | C-1003 |
| **Date of injury:** | March 13, 2024 |
| **First visit:** | March 15, 2024 |
| **Last visit:** | May 26, 2024 |
| **Number of visits:** | 24 |

### Services

| Service | Units | Charge |
|---|---|---|
| 99203 New patient evaluation | 1 | $683.95 |
| 98941 Spinal manipulation | 24 | $3,480.75 |
| 97110 Therapeutic exercise | 24 | $2,796.64 |
| 97014 Electrical stimulation | 12 | $1,346.23 |
| TOTAL | | $8,307.57 |

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## Treatment Summary

| | |
|---|---|
| **Patient:** | Tiffany Graham |
| **Treating provider:** | Kenneth Potter, DC |
| **Diagnosis:** | neck and back strain |

Patient presented following a motor vehicle collision with complaints of cervical and lumbar strain, including neck stiffness, reduced range of motion, paraspinal muscle tenderness, and interimttent low back pain radiating into the gluteal region without neurological deficit. Initial examination revealed restricted cervical rotation and flexion, along with palpable muscle guarding in the upper trapezius and lumbar paraspinal musculature. Neurological screening was within normal limits.

Treatment consisted of a standard plan of chiropractic spinal adjustments directed at the cervical and lumbar segments, combined with soft tissue mobilization. A progressive therapeutic exercise program focused on postural correction, core stabilization, and cervical range of motion was introduced and advanced as tolerated. Electrical stimulation was applied to the affected paraspinal muscles for pain modulation and muscle relaxation during the active treatment phase.

The patient attended a total of 24 visits over the course of care. Progress was documented at each session with reassessment of range of motion, pain scores, and functional tolerance. Subjective and objective findings improved steadily and have now plateaued. The patient has reached maximum medical improvement, with residual symptoms that are stable and not expected to change significantly with further conservative care. Discharge to a independent home exercise program is recommended, with follow-up as needed for flare-ups.